Adult Airway

AR-01 | Airway & Respiratory | Approved

Approved native content. Beta application; agency release requirements still apply.

Quick flow

01 Airway / Breathing Adequate? [EMT / AEMT / Paramedic]

Assess rate, effort, patency, oxygenation, ventilation, and mental status.

02 Basic Maneuvers First [EMT / AEMT / Paramedic]

Position • suction • OPA/NPA • two-person BVM ± PEEP.

03 Foreign-Body Obstruction? [EMT / AEMT / Paramedic]

Use age-appropriate obstruction procedure and direct visualization when authorized.

04 Support Still Needed? [EMT / AEMT / Paramedic]

All levels: oxygen/BVM. AEMT/Paramedic: authorized NIPPV or BIAD. Paramedic: intubation when clinically indicated.

05 Oxygenation / Ventilation Effective? [EMT / AEMT / Paramedic]

Maintain SpO_2 at least 92% unless a condition-specific target applies; suspected TBI requires at least 94%. If ineffective, move to the failed-airway pathway.

06 Suspected TBI? [EMT / AEMT / Paramedic]

SpO_2 ≥94%; urgently correct <90% or inadequate ventilation. Initial/routine assisted $EtCO_2$ 35-45 mmHg. No routine hyperventilation. Clear active/imminent herniation only: brief $EtCO_2$ 30-35 with capnography, avoid <30, and expedite transport. Open TB-05.

07 Confirm + Reassess [EMT / AEMT / Paramedic]

Waveform $EtCO_2$ after ETT/BIAD, secure device, monitor, and notify destination.

Overview

- A secure airway is one that provides effective oxygenation and ventilation, not simply an advanced device.
- Begin with positioning, suction, basic adjuncts, and effective BVM ventilation before escalating.

Provider-Level Actions

Use the least invasive effective airway intervention and escalate only when oxygenation or ventilation remains inadequate.

EMT

- Position and open the airway, suction, insert basic adjuncts, provide oxygen, and perform effective two-person BVM ventilation with PEEP when indicated.
- Continue basic airway care when it maintains adequate oxygenation and ventilation; do not delay transport solely to place an advanced airway.

AEMT

- Perform all EMT care plus authorized BIAD placement and continuous waveform capnography once available. Endotracheal intubation is not included as an AEMT procedure in this protocol.
- After an unsuccessful BIAD attempt, do not repeat blind placement; continue effective BVM and request Paramedic airway support.

Paramedic

- Perform all prior care plus ETT, advanced airway selection, DAI when authorized, chest decompression when indicated, and surgical airway for cannot-oxygenate/cannot-ventilate failure.

- Use waveform capnography continuously and transition immediately to the failed-airway protocol when criteria are met.

Indications

- Adult with actual or threatened airway obstruction, inadequate oxygenation, inadequate ventilation, or inability to protect the airway.

Treatment steps

- Target adult ventilation at approximately 10-12 breaths/min and use $EtCO_2$ and clinical response to avoid hyperventilation.
- Before an advanced-airway attempt, assess for difficult BVM ventilation, difficult laryngoscopy, inability to place a BIAD, and potential surgical-airway need. Prepare backup equipment and a rescue plan before beginning.
- For suspected traumatic brain injury, continuously monitor oxygenation and ventilation. Aim for SpO_2 at least 94%; urgently correct SpO_2 below 90% or inadequate ventilation. The condition-specific TBI target takes priority over a lower general airway or post-ROSC oxygenation target.
- For suspected traumatic brain injury requiring assisted ventilation, use an initial/routine $EtCO_2$ target of 35-45 mmHg. Avoid routine hyperventilation. Only for clear active or imminent cerebral herniation, use brief controlled ventilation targeting $EtCO_2$ 30-35 mmHg with continuous waveform capnography while expediting definitive care; avoid overshooting below 30 mmHg.
- Prevent hypotension in suspected traumatic brain injury; do not use permissive hypotension. For adults, target SBP at least 110 mmHg, including age 70, with frequent reassessment and after treatment. Pediatric TBI blood-pressure targets require age-specific direction from Medical Control until the separately approved pediatric TBI target table is published; do not substitute adult targets or generic pediatric shock thresholds.

Warnings / precautions

- Failed airway includes inability to oxygenate at least 90% after an unsuccessful attempt, anatomy unsuitable for further attempts, or three total unsuccessful attempts.
- No more than three total intubation attempts by the most experienced Paramedic.
- Waveform capnography is mandatory after ETT and BIAD placement once available.
- Effective BVM is an acceptable definitive field airway; do not delay oxygenation or ventilation for intubation or repeated laryngoscopy.

Clinical pearls

- DOPE: displacement, obstruction, pneumothorax, and equipment failure.
- Manually stabilize the ETT during every move or transfer.

Related pathways / contacts

App links open the deployed version. Confirm its package version matches this PDF before use.

TB-05 Head Trauma - Open the shared TBI oxygenation, ventilation, herniation-rescue, and perfusion targets.

Source and references

Claiborne County EMS approved protocol manual. | Claiborne County EMS formulary, linked protocol pathways, and agency release guidance.

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